Batterjee Medical College
For Science and Technology

Psychiatry · Clinical Skills

History Taking in Psychiatry

The structured psychiatric interview — building rapport, taking a complete history, screening the major disorders, and assessing risk.

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Learning Objectives

Mapped to the BMC Knowledge / Skill / Value framework. Prerequisite: basic clinical communication skills.

Knowledge & understanding

- Define a mental disorder and how psychiatric diagnosis is made
- List the nine sections of a complete history
- Recognise organic red flags and medical mimics

Skills

- Set up and conduct an interview that builds rapport
- Screen the major disorders bilingually
- Perform a structured suicide and homicide risk assessment
- Manage the difficult interview

Values & attitudes

- Treat the interview as a THERAPEUTIC ACT, not only data collection
- Respect privacy, confidentiality and boundaries
- Accept that asking about suicide is a duty, not a risk

What is psychiatry?

Definition

- The medical specialty devoted to the study, diagnosis, treatment and prevention of mental disorders
- Covers affective, behavioural, cognitive and perceptual abnormalities

Origin of the word

- Coined by Johann Christian Reil in 1808
- Greek: psykhē (soul) + iatreia (healing)
- Literally, the "medical treatment of the soul"

What is a mental disorder?

A syndrome of clinically significant disturbance in cognition, emotion regulation or behaviour, reflecting a dysfunction in the psychological, biological or developmental processes underlying mental functioning.

Usually involves

- Significant DISTRESS or DISABILITY
- In social, occupational or other important areas of functioning

NOT a disorder

- An expectable or culturally-approved response to a stressor or loss — e.g. bereavement
- Socially deviant behaviour, or conflict between the individual and society — UNLESS it results from a dysfunction in the individual

The exclusions matter as much as the definition — they are what stop psychiatry being used to pathologise difference or dissent.

Diagnosis is syndrome-based

Syndromes

- Current diagnoses rely on signs and symptoms that CO-OCCUR in groups of patients
- With a characteristic course and response to treatment
- A syndrome = a cluster of symptoms with a typical course and outcome

The two classification systems

- DSM-5-TR (APA, 2022)
- ICD-11 (WHO) — in effect since 2022, superseding ICD-10

There is no blood test or scan that diagnoses most psychiatric disorders — THE HISTORY AND MENTAL STATE EXAM ARE THE INVESTIGATION.

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PART 1

The Interview

Goals · setting up · technique · the framework of a complete history.

Goals of the psychiatric interview

Clinical goals

- Obtain the information needed to make a diagnosis
- Understand the PERSON and their circumstances, not just the illness

Relational goals

- Form a therapeutic relationship — rapport
- Give the patient information about the illness, the plan and the prognosis

The interview is simultaneously a diagnostic tool and a therapeutic act — rapport built here shapes everything that follows.

Setting up the interview

Set the scene

- Professional dress; introduce yourself and greet the patient by name
- Reassure about privacy and confidentiality
- Private, comfortable setting; L-SHAPED SEATING at a suitable distance — not face-to-face, which feels like an interrogation
- Explain who you are, the purpose, and the time needed

Conduct

- Start with OPEN-ENDED questions, then focus
- Observe non-verbal behaviour; avoid excessive note-taking
- Decide who to start with — patient or relative — and ask why a relative came

Structure of a complete psychiatric history

Follow the same order every time, so nothing is missed.

01 · Personal data

Name, age, marital status, religion, residence, occupation, handedness.

02 · Presenting complaint

In the patient's own words — and from a reliable informant.

03 · History of present illness

Onset, precipitant, symptoms present AND absent, help-seeking, response so far.

04 · Past psychiatric history

Admissions, episodes, ECT, diagnoses, treatment responses.

05 · Past medical & drug

Medical, surgical, and the psychiatric side-effects of medications.

06 · Substance history

Each substance: onset, pattern, route, effects, withdrawal.

Structure of a complete psychiatric history

The last three sections — the background that explains why this person, and why now.

07 · Family history

Health, relationships, mental illness, suicide, treatment response.

08 · Personal & developmental

Prenatal to adult life — milestones, school, work, marital, forensic.

09 · Premorbid personality

What the person was like BEFORE they became unwell.

Then: Mental State Examination → physical examination → investigations → formulation.

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PART 2

The Psychiatric History

Complaint and HPI · disorder screening (bilingual) · past, family, social and developmental history · premorbid personality.

Personal data & presenting complaint

Personal data & complaint

- Name, age, marital status, religion, residence, occupation, handedness
- The presenting complaint IN THE PATIENT'S OWN WORDS
- And from a reliable informant

Features of a reliable informant

- Close to, and living with, the patient
- No conflict of interest
- No psychiatric disorder themselves

Collateral history is essential in psychiatry — especially where insight is limited: mania, psychosis, dementia.

Analysing the current episode

Build the story

- Onset, and any stressor or precipitant
- Psychiatric symptoms PRESENT — and their full analysis
- Symptoms ABSENT — to exclude the differential diagnoses

Then ask about

- The way the patient sought help
- Response to treatment so far

Deliberately asking about ABSENT symptoms is what turns a history into a differential diagnosis.

Screening questions — GAD, panic & agoraphobia

Generalised anxiety

"Do you worry a lot about everyday things? Do you have trouble controlling your worries? Do friends say you worry too much?"

هل تقلق على أشياء كثيرة كل يوم؟ هل تجد صعوبة في التحكم في هذا القلق؟ هل الأهل أو الأصحاب يقولون إنك تقلق بكثرة؟

Panic attack

"Some people get a sudden surge of fear peaking within minutes — with palpitations, shortness of breath, chest pain, a choking feeling, a fear of dying, sweating and shaking?"

بعض الناس تجيهم نوبة نسميها نوبة هلع، وهي شعور مفاجئ بالخوف يزيد خلال دقائق، ويصاحبها خفقان أو كتمة وألم في الصدر وشعور بالخنقة مع تعرق ورجفة؟

Agoraphobia

"Have you ever avoided a place or situation for fear you might panic there — crowds, being outside alone, or public transport?"

هل تجنبت أماكن بسبب الخوف من أن تجيك نوبة فزع أو قلق شديد؟ مثل الأماكن المزدحمة أو الخروج لوحدك أو المواصلات؟

Screening questions — social anxiety & phobia

Social anxiety

"Do you get intense fear when you are the centre of attention — giving a speech, at a gathering, or facing an authority figure?"

بعض الأشخاص يجيهم قلق شديد لما يكون التركيز عليهم، مثل إلقاء كلمة أو الحديث في جمع أو مقابلة المدراء؟

Specific phobia

"Do you feel out-of-proportion fear of (e.g. animals) that interferes with your daily life?"

هل تشعر بخوف أو قلق من)الحيوانات مثلاً (بطريقة أكثر من المتوقع وبشكل يؤثر على حياتك وأعمالك اليومية؟

The feared CONSEQUENCE — not the situation — separates these disorders. Always follow a positive screen by asking WHY the situation is avoided.

Screening questions — OCD & psychosis

OCD

"Do you have symptoms like needing to wash your hands all the time because you feel dirty, constantly checking things, or annoying thoughts popping into your head over and over?"

هل لديك أعراض الوسواس القهري، مثل الحاجة لغسل اليدين معظم الوقت بسبب الشعور بعدم النظافة، أو فحص الأشياء بشكل متكرر، أو أفكار مزعجة تتكرر عليك؟

Psychosis

"Do you ever feel things are not real?" · "Do you worry that people might be against you or after you?" · "Do you ever hear or see things other people don't?" · "Are the voices outside or inside your head?" · "Do they ever command you to do things?"

هل تشعر أحياناً أن الأشياء غير حقيقية؟ هل تخاف أن الناس ضدك أو يتربصون بك؟ هل تسمع أو ترى أشياء لا يراها الآخرون؟ هل الأصوات من داخل رأسك أم من خارجه؟ هل تأمرك بعمل أشياء؟

Auditory hallucinations are more often experienced as coming from OUTSIDE, and patients may look toward the voice. ALWAYS ask about command hallucinations — it is a safety issue.

Past psychiatric, medical & drug history

Past psychiatric history

- Number of lifetime admissions and total episodes
- Ever had ECT or neurostimulation? Who is the treating provider?
- Diagnoses, treatments, and the response to each

Medical, surgical & drug history

- Medical and surgical history
- Psychiatric SIDE-EFFECTS of medications — steroids, interferon, some antihypertensives

A long, well-documented past history is best captured by counting admissions and episodes and noting treatment responses, rather than a blow-by-blow narrative.

Substance history & CAGE

CAGE

For EACH substance ask: age of onset · amount and pattern · route · effects · withdrawal. Tobacco (packs/day, NRT) · opioids (type, IV/PO, naloxone) · alcohol (blackouts, binges, withdrawal, seizures) · cannabis (paranoia?) · stimulants · benzodiazepines · and CAFFEINE, which is key if the patient is anxious.

C Cut down

Have you ever felt you should cut down on your drinking?

A Annoyed

Have people annoyed you by criticising your drinking?

G Guilty

Have you ever felt guilty about your drinking?

E Eye-opener

Have you ever had a drink first thing in the morning to steady your nerves?

CAGE screens for alcohol use disorder — ≥ 2 positive answers is a concern and warrants a full assessment.

Family history

The family

- Father, mother, siblings — health and relationships
- Home atmosphere; monthly income; crowding

Family psychiatric history

- Mental illness, substance use, personality disorder
- COMPLETED SUICIDE
- Response to specific treatments — this guides drug choice

A strong family history raises the pre-test probability of mood and psychotic disorders — and a relative's treatment response can guide the patient's own.

Personal & developmental history

Early life

Prenatal, natal and postnatal history · developmental milestones · childhood "neurotic traits".

Growing up

Scholastic progress · work record.

Adult life

Sexual and marital history · military or national service · forensic history.

A poor school and work record from early on points to something lifelong rather than a new illness — which is what the next slide tests.

Premorbid personality

What was the person like BEFORE they became unwell? — the baseline against which the illness is measured.

Domains to cover

- Sociability and relationships
- Impulse control
- Emotional stability
- Prevailing mood

...continued

- Character of thinking
- Religiosity
- Habits and hobbies
- Interests

A marked CHANGE from premorbid personality is itself a red flag — and it helps separate a personality disorder (lifelong) from a new illness (a change).

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PART 3

Risk Assessment

Asking about suicide · current safety and past attempts · risk factors · homicide.

Which factor most raises the risk?

You are assessing a depressed patient and weighing their suicide risk.

Four features are present in different patients you see today.

Which most strongly predicts suicide?

A Feeling stressed at work

B A previous suicide attempt

C Being married

D Female sex

Commit to an answer before the next slide.

Which factor most raises the risk?

ANSWER

You are assessing a depressed patient and weighing their suicide risk.

Four features are present in different patients you see today.

Which most strongly predicts suicide?

- **A** Feeling stressed at work
- **B** **A previous suicide attempt**
- **C** Being married
- **D** Female sex

B — a previous suicide attempt.

Past attempts are the single strongest predictor of future suicide. Marriage is protective; completed suicide is higher in males despite more attempts in females.

How to ask about suicide

Normalise, then ask directly

- "Some people think of suicide when their mood is low — has this ever crossed your mind?"
- "You're going through a painful loss — have thoughts of your own death ever crossed your mind?"

The myth — dispelled

- Asking about suicide does NOT plant the idea or increase risk
- It RELIEVES distress and opens the conversation
- Most patients are relieved to be asked

Screening for suicidal — and homicidal — ideation is MANDATORY in every assessment. Never leave it out.

Current safety & past attempts

Current — the "how serious"

- Is there a PLAN?
- Access to MEANS — firearms, medications, poisons?
- Intent to act immediately?
- Chronic, acute and imminent risk factors?

Past — the "index attempt"

- Ask about any previous attempt: when, how, why?
- Was it carried out with expressed INTENT TO DIE, as opposed to an accidental overdose?
- Any self-harm that could cause an "inadvertent suicide"?

Suicide risk & protective factors

SUICIDE RISK — WHAT TO WATCH, WHERE TO ACT

STATIC — fixed
flag who to watch

- **PREVIOUS ATTEMPT — the strongest**
- Male sex · older age
- Family history of suicide
- Chronic illness · past trauma

DYNAMIC — modifiable
where you intervene

- Active depression or psychosis
- Hopelessness · agitation
- Substance use or intoxication
- **ACCESS TO MEANS** · recent loss · isolation

PROTECTIVE
strengthen these

- Social support · dependent children
- Faith and religious belief
- Engagement with treatment
- Future orientation

Risk is DYNAMIC — reassess it. Asking about suicide **does not plant the idea** — it relieves distress.

Homicide & risk to others

Ask directly

- Threats to others driven by psychotic symptoms — command hallucinations, persecutory delusions?
- Threats to SPECIFIC, NAMED individuals?
- "If you left now, would you want to hurt anyone?"

Consider

- Access to weapons; history of violence
- Duty to warn or protect an identifiable target — under the local legal framework

Homicide risk is screened in every assessment, not only when something in the presentation prompts it.

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PART 4

Special Situations

Organic red flags and medical mimics · psychiatric side-effects of drugs · the difficult interview.

New "psychiatric" symptoms at 58

A 58-year-old with no psychiatric history develops, over two weeks, visual hallucinations, fluctuating confusion that is worse at night, disorientation, and a mild fever.

He has never been unwell like this before.

Your first priority?

A Start an antipsychotic for psychosis

B Investigate for an organic / medical cause

C Refer for psychotherapy

D Diagnose late-onset schizophrenia

Commit to an answer before the next slide.

New "psychiatric" symptoms at 58

ANSWER

A 58-year-old with no psychiatric history develops, over two weeks, visual hallucinations, fluctuating confusion that is worse at night, disorientation, and a mild fever.

He has never been unwell like this before.

Your first priority?

A Start an antipsychotic for psychosis

B **Investigate for an organic / medical cause**

C Refer for psychotherapy

D Diagnose late-onset schizophrenia

B — investigate for an organic cause.

Late onset, no prior history, visual hallucinations, fluctuating consciousness and fever scream delirium, not a primary psychiatric disorder. Find and treat the cause.

Organic red flags

The presentation

- New onset over about 40 with NO prior psychiatric history
- Fluctuating consciousness or disorientation — delirium
- VISUAL, olfactory or tactile hallucinations
- Focal neurological signs; seizures; abnormal vital signs

The course

- Rapid, atypical, or treatment-resistant course
- Cognitive decline out of keeping with the mood
- Always take a full medical and drug history, and EXAMINE the patient

The golden rule: exclude an organic cause BEFORE diagnosing a primary psychiatric disorder — especially with late onset or atypical features.

Medical conditions that mimic psychiatric illness

Endocrine / metabolic

- Thyroid disease; Cushing's, Addison's
- Hyper- or hypocalcaemia, PTH
- Low Na / K / Mg; hepatic encephalopathy; porphyria

Neurological

- Epilepsy, especially temporal lobe
- MS, Huntington's, traumatic brain injury
- Kleine–Levin, Klüver–Bucy

Infective / inflammatory

- Autoimmune (anti-NMDA) and Hashimoto's encephalitis
- Neurosyphilis, HIV, Lyme, cryptococcal meningitis
- SLE; paraneoplastic syndromes

Drugs / other

- Medication and substance effects
- Delirium; catatonia
- Niemann–Pick C; Moyamoya; post-MI

You are not expected to memorise every entry — you are expected to reach for this list whenever the red flags on the previous slide are present.

The difficult interview

Situation	Approach
Guarded / suspicious	Go slowly, be transparent, do not challenge delusions; build safety first
Hostile / agitated	Calm and non-confrontational; ensure your own safety and an exit; set limits
Mute / withdrawn	Patience, open posture, collateral history; consider catatonia or depression
Over-talkative	Gently structure and redirect with closed questions
Seductive / boundary-testing	Maintain professional boundaries; use a chaperone; document

These are examined in OSCEs and met on every ward — the common thread is that you manage the interaction rather than simply pushing on with your question list.

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PART 5

Completing & Review

Physical examination · closing the interview · a worked case · board questions · take-home.

Physical examination

What to do

- General examination, including vital signs
- Neurological — motor, sensory, reflexes, and FUNDUS
- Cardiac and chest examination

Why it matters

- Screens for the organic causes just discussed
- Provides a BASELINE before starting medication — weight, blood pressure, ECG

Psychiatry is a branch of medicine — a physical examination is part of a complete psychiatric assessment, not an optional extra.

Closing the interview

Check & collaborate

- "Did we cover the main concerns you hoped to discuss today?"
- "Do you have any thoughts on how we might be able to help?"

Finish with respect

- Summarise and agree the plan
- Thank the patient for their time, and for sharing a "snippet" of their life

A collaborative close reinforces rapport and adherence — the interview began, and ends, as a therapeutic act.

Mr. S, 62 — brought in by his son

Mr. S is a 62-year-old retired teacher, brought to clinic by his son. He says nothing is wrong.

The son reports that over the past three weeks his father has become suspicious that the neighbours are entering the house, and has twice said he saw children sitting in the living room who were not there. He is more confused in the evenings and sleeps poorly, but some mornings seems almost himself.

He has no psychiatric history. He was started on prednisolone six weeks ago for a chest problem, and takes tramadol for back pain. His son thinks he has been drinking less water than usual and looks like he has lost weight.

On arrival he is mildly febrile, unsure of the date, and asks you twice why he is in the clinic.

Case problem — work through it

Questions

- 1 · Whose account will you rely on, and what makes an informant reliable?
- 2 · Which features here are organic red flags?
- 3 · What is your working diagnosis, and what must you exclude?
- 4 · Which parts of the history and examination do you prioritise now?
- 5 · What risk must you still assess before he leaves?

Model answer

- 1 · The son — collateral history is essential when insight is lost. He is close to and lives with the patient, has no conflict of interest and no psychiatric disorder of his own
- 2 · New onset at 62 with no history · visual hallucinations · fluctuating consciousness, worse at night · disorientation · fever
- 3 · DELIRIUM — organic until proven otherwise. Do not diagnose late-onset psychosis. Exclude infection, drugs (prednisolone, tramadol), dehydration and electrolyte disturbance
- 4 · Full medical and drug history, substance history, then physical examination — vitals, neurological including fundus, chest — plus investigations
- 5 · Suicide AND homicide risk — his persecutory beliefs about the neighbours make risk to others a live question; ask directly

Finish by asking what would change if the same picture had developed over three years rather than three weeks — the answer being a dementia, and a different work-up.

Board-style questions

#	Question	Answer
1	Single strongest predictor of completed suicide?	A previous suicide attempt
2	Does asking about suicide increase the risk?	No — it does not plant the idea, and it must always be asked
3	What does the CAGE questionnaire screen for?	Alcohol use disorder (≥ 2 = concern)
4	New-onset visual hallucinations plus fluctuating consciousness at 60 — think what?	Delirium / an organic cause — investigate before diagnosing psychiatric illness
5	Features of a reliable informant?	Close to and living with the patient, no conflict of interest, and no psychiatric disorder

Summary — six things to remember

The method

- The history and MSE ARE the investigation — rapport is the foundation
- Start open-ended; ask about ABSENT symptoms to build a differential
- Always take a collateral history from a reliable informant

The safeguards

- Always assess suicide AND homicide risk — asking does no harm
- Previous attempt = the strongest suicide predictor
- Late onset, atypical or fluctuating → exclude an organic cause first

References

Course official textbooks

- Kaplan & Sadock's Synopsis of Psychiatry
- Oxford Handbook of Psychiatry
- DSM-5-TR (APA, 2022); ICD-11 (WHO)

Others & tools

- CAGE questionnaire (alcohol)
- Standard suicide and violence risk-assessment frameworks
- Bilingual screening scripts adapted for Saudi practice
- Classification updated to DSM-5-TR and ICD-11, current since 2022

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Thank You

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Questions welcome. Sources for every figure are on the references slide.